

Developer Briefing Document

Care Minutes AI — MVP for Australian Aged Care

Version 1.0 · April 2026 · For Development Team

Purpose: This document explains everything a developer needs to understand about the problem we are solving, the people we are building for, what the product must do, and how the team works together — with zero assumed knowledge of the Australian aged care industry.

1. The Problem — In Plain English

1.1 What is aged care?

Aged care in Australia means residential nursing homes where elderly people live when they can no longer look after themselves. These facilities provide day-to-day support: helping residents eat, bathe, move, take medication, and receive nursing care.

Australia has approximately 2,617 of these facilities, caring for roughly 245,000 elderly residents. They are run by a mix of government bodies, not-for-profit organisations, and private companies.

1.2 How are facilities funded?

Most aged care facilities receive the majority of their income from the Australian Federal Government. The government pays a daily subsidy per resident — called the AN-ACC (Australian National Aged Care Classification) subsidy. In simple terms: more residents with higher care needs means more government money.

Critical insight: This government subsidy is not optional income — it is the primary revenue source for most aged care providers. Losing even a fraction of this funding can be financially devastating for a small facility.

1.3 What are care minutes?

Care minutes is a government regulation that requires every resident in a facility to receive a minimum amount of direct care time from qualified staff every single day. As of 2026:

Rule	What it means
215 minutes total	Every resident must receive at least 215 minutes of direct care per day, averaged across all residents in the facility
44 minutes RN minimum	Of the 215 total minutes, at least 44 must be delivered specifically by a Registered Nurse (RN)
Applies every day	This is not a weekly average — compliance is measured daily, 365 days per year
All residents counted	The target applies to all permanent and respite residents in the facility

Example: A facility with 40 residents must deliver $40 \times 215 = 8,600$ minutes of care per day — roughly 143 hours of qualified staff time, every single day.

1.4 What happens when a facility misses the target?

From April 2026, the government introduced direct financial penalties for non-compliance:

Penalty: Up to \$31.64 per resident per day is deducted from government funding for every day a facility fails to meet its care minutes target.

For a facility with 40 residents missing its target for an entire quarter (90 days):

Factor	Amount
Residents	40
Penalty per resident per day	\$31.64
Days in a quarter	90
Total quarterly penalty	\$113,904

Beyond the financial penalty, facilities that repeatedly fail are listed publicly on the MyAgedCare government website — visible to families choosing a care home, destroying the facility's reputation.

1.5 Why is this hard to track today?

Most small aged care facilities currently have no reliable real-time way to know if they are meeting their targets. Here is what typically happens today:

1. A manager manually totals staff shift hours in a spreadsheet — usually at the end of the week or quarter.
2. They calculate compliance manually — a process taking hours due to different staff types, part-time workers, and agency staff.
3. If they discover a shortfall, it is often too late — the quarter has ended and the penalty is already locked in.
4. For quarterly audits, they spend 1–2 full days compiling documents and spreadsheets to submit to the government.

The result: facilities fly blind — discovering compliance failures after the money is already lost, and spending enormous time on manual admin that could be automated in seconds.

2. Who We Are Building For

2.1 Our target customer

We are building for small aged care providers — facilities with under 60 residents. There are approximately 838 such facilities in Australia. This segment is our initial focus because:

- They are underserved — enterprise software exists for large providers but is too complex and expensive for small ones
- They make purchasing decisions fast — one manager can say yes without a procurement committee
- They feel the most pain — fewer resources to manage compliance manually
- They pay by credit card — no lengthy IT procurement required

2.2 The primary user

The main person using our product daily is the Director of Nursing (DON) or Operations Manager at a small facility. Understand this person deeply:

Attribute	Detail
Their job	Managing 20–60 care staff, overseeing clinical compliance, rostering, handling family complaints, managing audits — all at the same time
Tech skills	Moderate — comfortable with email and spreadsheets. Not a technical user. Will not read a manual.
Biggest fear	Receiving a government letter saying they missed care minutes and owe back a large sum of funding
When they work	Early mornings and late evenings — often checking things on a mobile phone, not at a desktop computer
What they want	To know in 10 seconds every morning whether their facility is on track — and exactly what to do if it is not

2.3 Before vs after our product

Before our product — a typical morning:

Time	What happens
7:00am	Arrives. Checks paper handover notes. Has no idea if yesterday met the care minutes target.
8:30am	Two staff call in sick. Spends 40 minutes calling casuals from memory or a paper list.
11:00am	Opens a spreadsheet to check this week's care minutes tally. Takes 40+ minutes manually.

11:45am	Discovers the facility is 18 minutes short this week. Too late to easily recover.
Quarter end	Spends 2 full days compiling the compliance audit report from spreadsheets.

After our product:

Time	What happens
7:00am	Opens the app on her phone. Green dashboard. One alert: "Add 1 RN shift Thursday to stay on track."
7:05am	Calls the suggested RN from the staff list. Shift filled. Done.
Quarter end	Clicks Generate Report. Downloads a formatted PDF. Emails it to the auditor. 2 minutes total.

This transformation — from 40 minutes of manual spreadsheet work to a 10-second morning check — is the core value we deliver. Keep this in mind when making any design or feature decision.

3. The Product We Are Building

3.1 Purpose

A web-based SaaS application that gives small aged care providers real-time visibility into their care minutes compliance status — and proactively tells them exactly what to do to avoid financial penalties.

Working product name: CareMinutes.ai (final name TBD)

3.2 What we are NOT building in MVP

Out of scope	Why
Full rostering system	Managers enter shifts manually — no drag-and-drop roster builder
Mobile app (iOS/Android)	A mobile-responsive web app — no app store submission
Payroll processing	We export data for payroll — we do not process pay
Clinical notes or care plans	Different product, different buyer — out of scope
Full ACQS compliance suite	Broader than care minutes — out of scope for MVP
System integrations	No API connections in MVP — all data entered manually

3.3 The 4 core features

FEATURE 1

Live Care Minutes Tracker

A single dashboard showing today's compliance status. The manager opens this and immediately knows if they are safe or at risk.

Must show:

- Today's total care minutes delivered vs the 215-minute target — number, progress bar, and RAG status
- Today's Registered Nurse minutes delivered vs the 44-minute minimum
- RED if below 85% of target, AMBER if 85–99%, GREEN if 100% or above
- 14-day trend chart of daily compliance
- Breakdown by staff type: RN, EN, PCW, Agency

CRITICAL: The compliance calculation is the most important logic in the product. A bug here directly affects how much government money a facility receives. Write comprehensive unit tests covering every edge case before building anything else on top of it.

FEATURE 2

Quarterly Forecast + Penalty Calculator

A forward-looking view showing whether the facility is on track for its quarterly target — and the dollar value of funding at risk if current pace continues.

Must show:

- Current quarter compliance percentage based on actual minutes logged
- Projected quarter-end result if current pace continues
- Dollar value at risk: $(\text{shortfall minutes} \div 215) \times \text{residents} \times \$31.64 \times \text{days remaining}$
- "You need X minutes per day for the rest of this quarter to recover"
- Scenario tool: "If I add X shifts per week, will I make the target?"

Why this matters: When a manager sees "\$47,200 at risk this quarter," the \$800/month subscription cost becomes an obvious decision. This feature closes sales.

FEATURE 3

AI Shift Gap Alert

Every morning at 7:00am AEST, the system analyses shift data and sends a plain-English alert telling the manager exactly what action to take that day.

The alert must contain:

- Clear status: "You are on track this week" OR "Action needed to stay compliant"
- If action needed: specific days, staff types, and minutes each additional shift would contribute
- Suggested staff to contact, prioritised by qualification and availability

How the AI works: We call the Claude API (claude-sonnet-4-6) with the facility's shift data as context. The prompt asks Claude to identify the compliance gap, determine which shift types fill it most efficiently, and suggest staff from the facility list who are likely available. This is structured data analysis with a natural language output layer — not complex AI.

Delivery: Email (Resend) and in-app notification. SMS for urgent breach warnings (Growth plan only).

FEATURE 4

One-Click Audit PDF Report

The manager clicks one button and receives a professionally formatted PDF compliance report for any date range — used for the quarterly ACQSC audit submission.

The report must include:

- Cover page: facility name, reporting period, compliance result (met/not met), generated date
- Summary table: total minutes, target, compliance %, RN coverage days met
- Daily breakdown: one row per day with minutes, target, and status
- Staff type breakdown: hours by RN, EN, PCW, Agency
- Trend chart: daily compliance over the quarter
- Agency vs permanent split percentage

Why this matters: Providers currently spend 1–2 days compiling this manually. We reduce it to 10 seconds. This alone justifies the subscription for many managers.

4. Compliance Rules — Implement Exactly

These rules come directly from Australian Department of Health legislation. Implement them exactly as written. Do not interpret or approximate.

Rule	Specification
Daily total target	215 minutes per resident per day, averaged across ALL residents in the facility
RN daily minimum	44 minutes of the 215 must be delivered by a Registered Nurse specifically
Qualifying staff	RN, EN, and PCW all count toward the 215-minute total
RN-only rule	Only RN time counts toward the 44-minute minimum — EN and PCW time does not qualify
Measurement period	Daily — not weekly or monthly average
Penalty trigger	Up to \$31.64 per resident per day reduction in Base Care Tariff for non-compliant days (from April 2026)
Agency staff	Counts toward total care minutes but must be tracked and reported separately from permanent staff
Quarterly reporting	Providers submit a Quarterly Financial Report (QFR) showing care minutes broken down by staff type

5. Glossary of Aged Care Terms

Every developer on this project should read this glossary before writing a single line of code.

Term	Definition
AN-ACC	Australian National Aged Care Classification — the government funding model that determines daily subsidy per resident
ACQSC	Aged Care Quality and Safety Commission — the government body that audits facilities and issues penalties
QFR	Quarterly Financial Report — mandatory quarterly submission providers make to the government showing care minutes data
RN	Registered Nurse — highest qualification, requires a university degree and AHPRA (national nursing board) registration
EN	Enrolled Nurse — TAFE diploma, lower qualification than RN but still counts toward 215-minute total
PCW	Personal Care Worker — entry-level care staff, no formal nursing qualification, counts toward 215-minute total
DON	Director of Nursing — most senior clinical person at a facility, our primary product user
Roster	The schedule of which staff member works which shift on which day
Shift	One block of work time — e.g. "Sarah Chen, RN, Monday 7am–3pm = 480 minutes"
Care minutes	Total minutes of direct care delivered to residents on a given day
Agency staff	Temporary workers from a staffing agency, used to fill gaps — tracked separately in compliance reports
MyAgedCare	Government public website where families can view a facility's compliance records
Base Care Tariff	The core daily government payment per resident — reduced when care minutes are not met
Star Rating	Government quality rating published publicly — poor care minutes compliance reduces this rating

6. How We Work Together

6.1 6-week build plan

Week	Focus
Week 1	Project scaffold, authentication (Clerk), Supabase database setup, staff profiles CRUD, shift entry UI
Week 2	Shift management, weekly view, CSV bulk import for existing staff lists
Week 3	Compliance calculator engine — daily minutes, RN split, quarterly forecast, unit tests. Most critical week.
Week 4	Dashboard UI — live tracker, RAG status, trend chart, penalty calculator, quarterly forecast screen
Week 5	AI alert engine — 7am daily job (Inngest), Claude API integration, email delivery (Resend), in-app alerts
Week 6	PDF audit report, multi-tenant hardening, Stripe billing, onboarding wizard, production deployment

6.2 Ticket format

Every task will be a detailed ticket. Before starting any feature, you must be able to answer: Who needs this and why? What does "done" look like exactly? What are the edge cases? Is there a compliance rule that affects this?

Example ticket: Daily compliance calculator

User story: As an operations manager, I need to see today's total care minutes and RN minutes so I know instantly if I am on track.

Acceptance criteria:

1. Dashboard shows total minutes = sum of all shift durations for that date
2. RN minutes = sum of durations where staff.role = RN only
3. Compliance % = $(\text{total_minutes} / (\text{facility.resident_count} \times 215)) \times 100$
4. Status = RED if < 85%, AMBER if 85–99%, GREEN if ≥ 100%
5. Calculation updates within 2 seconds of a new shift being saved

Edge cases: Handle zero shifts (show 0 not null). Handle public holidays same as any other day.

6.3 Team rhythm

When	What
Monday	30-min sprint planning call — founder sets 3 priorities for the week
Daily	Async standup in Slack — what you completed, what you're working on, any blockers
Wednesday	20-min mid-week check call — on track? Any decisions needed?
Friday	30-min demo — each dev shows what they built, founder gives feedback
Timezone	India (IST) is 5.5 hours behind Australia (AEST). Best live overlap: 9–11am IST = 2:30–4:30pm AEST

6.4 Non-negotiable quality rules

5. Compliance calculator must have unit tests covering all edge cases before any other feature builds on top of it
6. All Supabase data stored in Sydney region (ap-southeast-2) — Australian Privacy Act requires resident data in Australia
7. Row Level Security enabled on all tables — one facility must never see another facility's data
8. Care minutes targets must come from the database, not be hard-coded — regulations can change
9. Every page must be mobile-responsive — managers check the app on phones
10. All code reviewed via pull request before merging to main branch

Final note: The people using this product are under real financial and regulatory pressure. A bug in the compliance calculator could cause a facility to believe they are compliant when they are not — leading to real financial penalties for real people. Build this with that responsibility in mind.